

Musculoskeletal Claims

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Agenda



- Some facts and figures / financial impact of back pain
- The spine and pain
- Undertaking a holistic assessment
- Rehabilitation
- Permanence?

Facts and Figures



- Low back pain is common and affects around 1/3 of the UK adult population each year. Around 20% of people with low back pain (around 1 in 15 of the population) will consult their GP about it
- According to a survey published in 2000 almost half the adult population of the UK (49%) report low back pain lasting for at least 24 hours at some time in the previous year
- It is estimated that almost 5 million working days are lost annually through 'work related back pain'
- Four out of every five adults (80%) will experience back pain at some stage in their life, although in most cases back pain is nothing serious and disappears spontaneously, the sheer number of people affected makes it a very costly condition imposing a considerable burden on the individual and society.

An expensive business...



- Treating all types of back pain costs the NHS more than £1000 million per year (NICE 2009)
- Lost production, as a result of low back pain, costs at least £3500 million per year (Maniadakis and Gray 2000)
- Musculoskeletal problems were responsible for 20% of UK absence in 2013 – 30.6 million days (ONS 2014)
- In the top 3 causes of absence for Income Protection (with psych and cancer)



What can go wrong?



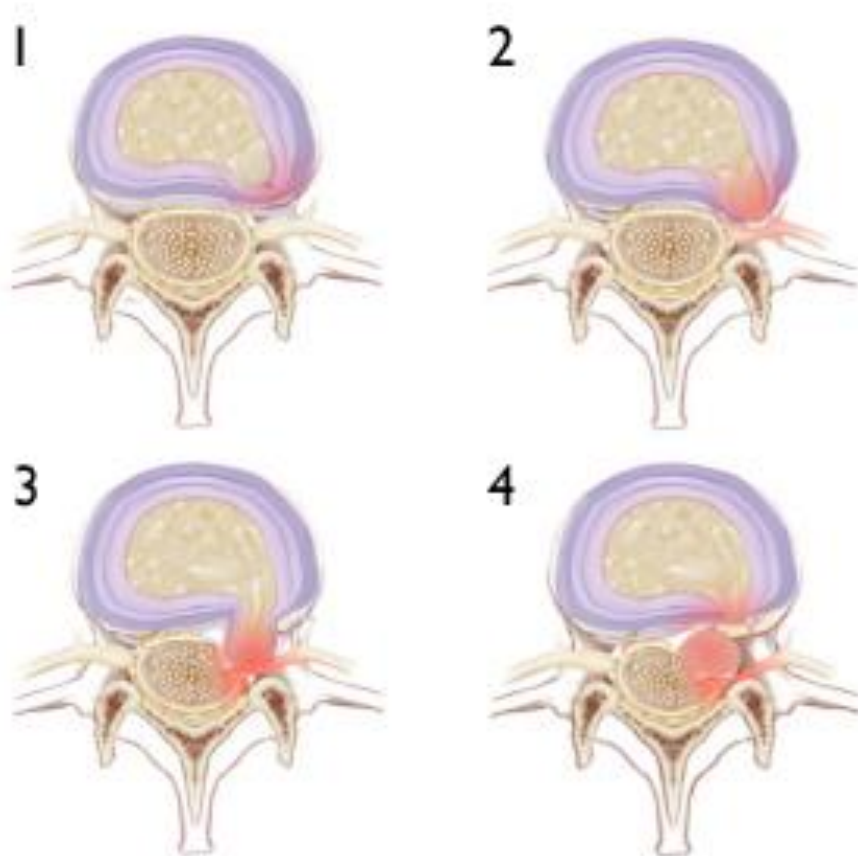
- The lower back is subject to a lot of mechanical stress and strain
- The spine, made up of more than 30 small bones (vertebrae) stacked on top of each other, supports all the upper body weight which loads the lower back
- A spongy piece of cartilage (disc) sits between each vertebra acting as a shock absorber and preventing the vertebrae from grinding against each other
- With age, the discs wear and shrink (degenerative disc disease). They can tear as a result of injury
- The weakened disc can put pressure on its' jelly like centre, which can cause herniation
- Herniated discs can press on the nerves causing neuropathic pain including pain into the buttocks and down the leg (sciatica)

What the Fit Note may say..



- Low back pain
- Work related back pain
- Disc herniation (slipped disc, ruptured disc)
- Degenerative disc disease
- Sciatica
- Disc tear
- Spondylolisthesis
- Osteoarthritis
- Spinal stenosis
- Scoliosis
- Facet joint syndrome
- +++

'Slipped Disc'



1. The jelly centre tears the outer fibres that hold the disc in place. Pain can be caused as some of the layers tear.
2. The jelly has pushed the outer layers out into a bulge causing inflammation and irritation of the nerve (but not compression).
3. The prolapse is pressing on the exiting nerve so symptoms may now increase or radiate down the leg
4. Most severe but least common, the jelly has broken through the layers and is free floating, compressing most of the nerve. A medical emergency

Some definitions



Acute Pain - Short term pain of less than 12 weeks duration

Chronic Pain – Continuous pain of over 12 weeks duration or continuing beyond the point that healing would be expected following trauma or surgery

Neuropathic Pain – Pain initiated or caused by a primary lesion or dysfunction in the peripheral or central nervous system e.g. spinal cord trauma or impingement

Considerations for Assessment



- Early Intervention
- Review of medical evidence
- Holistic assessment of the individual
- Understand the job
- Engage with employer
- Look at opportunity for early return to work
- Support until return to work achieved and sustained



Early Intervention



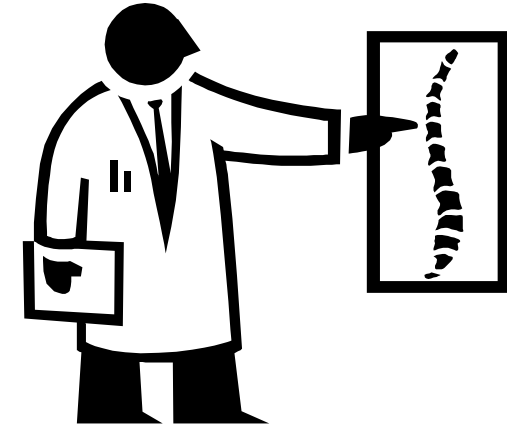
- Encourage early submission or notification of a claim or absence
- Focus on obtaining information in a timely manner
- Gather evidence and undertake assessments concurrently to expedite treatment and minimise risk of extended absence
- Consider weight of evidence in relation to IP policy
- Engage with the claimant and the employer to maintain momentum and motivation to RTW



Review of Medical Information



- History of condition
- Findings on clinical examination
- Specialist referrals
- Other medical conditions
- Investigations - X Rays, Magnetic Resonance Imaging (MRI), Computerized Axial Tomography (CAT) scan, Bone Scan, Bone Density
- Treatment plan – physical / psychological therapy, medication prescribed
- Are NICE guidelines being followed?
- What advice has been given to the patient / claimant?
- Prognosis



NICE Guidelines – CG88

Early Management of Persistent and Non-Specific Back Pain



Treatment.....

Offer one of the following treatment options

- An exercise programme
- A course of manual therapy
- A course of acupuncture

and if no response is shown to the treatment at early review, consider offering another of the options

Combined physical and psychological treatment programmes should be offered to individuals who have received at least one of the treatments above or who have a high level of disability or are displaying significant psychological distress.

NICE Guidelines – CG88

Early Management of Persistent and Non-Specific Back Pain



Assessment and Imaging.....

X Rays of the lumbar spine should not be offered for the management of non-specific low back pain

An MRI should only be offered within the context of a referral for an opinion on spinal fusion (which should **only** be considered once the individual has completed an optimal package of care, including combined physical and psychological treatment and still have severe non-specific back pain)

NICE Guidelines – CG88

Early Management of Persistent and Non-Specific Back Pain



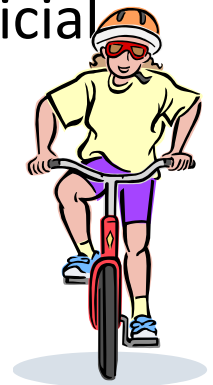
Information and education.....

Inform the patient about the nature of non-specific low back pain

Encourage the person to be physically active – this is likely to be beneficial

Encourage normal activities as far as possible

People with low back pain should be advised to exercise



Holistic Assessment

Psychological Flags

- Psychiatric history
- Alcohol / drug use, misuse, dependence or addiction
- Low mood – anxiety or depression
- Low confidence
- Poor motivation
- Fear
- Beliefs and assumptions
- Unrealistic expectations
- Acceptance of current situation



Social Flags

- Dissatisfaction with work
- Performance / relationship issues at work
- Secondary gain
- Influence of family on pain behaviours
- Interpersonal support
- General fitness
- Lack of engagement in treatment programme

Consider the full picture...



- Understand the medical
 - Review of notes
 - Has assessment, treatment and management met NICE Guidelines
 - If seeking independent advice be clear about the questions you want answered
- Understand the psycho social
 - Engage with the claimant
 - Engage with the employer
- Understand the function
 - Ask what the individual can and cannot do and why
 - Compare to job role
 - Consider a Functional Capacity Evaluation **EARLY** in the life of the claim – it may help signpost treatment

Job Roles



- Never assume understanding of what a job involves from the title
- Are job descriptions helpful in terms of job demands?
- The perception of the employee and employer regarding the role may differ
- Have adjustments been made, are any possible?
- The employer may not be aware they can seek specialist advice and financial support to put in place reasonable adjustments
- Consider a job demands analysis if in doubt



What can the individual not do and why can they not do it?

Look for the rehabilitation 'window of opportunity'



- Engage all parties – claimant, employer, OH and other treatment providers
- Discuss what reasonable adjustments may be helpful
- Develop a phased return to work that is 'fit for purpose'
- Assign ownership, roles and responsibilities
- Be careful not to medicalise the non medical!
- Build the case, backed up by NICE Guidelines and Equality Act 2010



Permanent disability?



Concluding back pain of a non-specific nature will result in permanent incapacity can be challenging.....

- Undertake a comprehensive and holistic assessment up front
- Exploring and challenging Psychological and Sociological factors following acceptance will be difficult
- Is the evidence supporting incapacity consistent and credible, and does it continue to be so?

A final thought.....



‘Someone who has been off sick for 6 months or longer has an 80% chance of being off work for 5 years...’

Waddell and Burton 2006

Any Questions?





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